



CHILD CARE PROFESSIONALS OF BC

A Pathway to Universal Child Care in BC by 2028

Policy proposals on stabilization, inclusion and universal funding

September 2026

Universal.

Every child. Every family. Every educator. Every community.

Summary

British Columbia now has one licensed child care space for every three- and four-year-old, and in more than half its school districts, more spaces than children. The spaces were built in anticipation of universal child care. What is missing is a funding model that can keep them open, make them affordable to every family, and staff them for the children who need support. With a smaller cohort of young children and slower immigration, operators across BC are reporting vacancy and under-enrolment, and six in ten operators responding to our survey say they can continue for two years or less, or are already planning a closure or reduction.

Child Care Professionals of BC (CCPBC) asks the Province to act in three linked steps: **stabilize the sector now, fund inclusion, and move to a universal funding model from April 1, 2028.**

Stabilize (Part 1)

1. **Pay operating funding on 75% of licensed spaces.** From April 1, 2027, pay CCOF base funding, the CCFRI fee reduction and the CCFRI provider payment on 75% of a facility's licensed spaces, or on actual enrolment, whichever is greater. Apply it to facilities with three or more children enrolled that have held a licence at their address for at least twelve months. End it when universal funding is in place, or on March 31, 2029, whichever comes first.

Include (Part 2)

2. **Require an inclusion policy, with a provincial obligation attached.** Issue a Director of Licensing Standard of Practice requiring every licensed program to hold a written inclusion policy covering enrolment, emergence and limits. At the limits stage, trigger a Supported Child Development referral with a published response time.
3. **Fund support, pay it to the provider, and remove the income test.** Replace the special needs supplement with a support payment to the operator in the range of \$750 per child per month, with the goal of matching K–12 levelled funding. Keep the existing special needs assessment as the gate, and use SCD consultants and short-term aide funding for children in the emergence period.
4. **Recognize the educators who do this work.** Add a certification differential to the ECE Wage Enhancement for Special Needs Educators and Infant Toddler Educators.
5. **Create an annual accessibility grant open to every licensed operator,** regardless of auspice or funding stream, with a published list of eligible items.

Go universal (Part 3)

6. **Fund every licensed centre at its regional benchmark, less one parent co-pay.** From April 1, 2028, the Province pays the published regional affordability benchmark less a single province-wide co-pay, estimated at \$15 to \$23 a day. The Affordable Child Care Benefit continues underneath for the lowest-income families.

- 7. Secure and grow the ECE Wage Enhancement.** Protect the existing \$6.00 an hour, add \$2.00 an hour each for ITE and SNE certification, develop a public regional wage grid in consultation that respects collective bargaining rights, and continue the Professional Development Bursary.
- 8. Set transparent operating-hour ranges** so benchmarks follow a published formula rather than centre-by-centre contract negotiation.
- 9. Consult with operators on the benchmark formula** during the 2027–28 stabilization year.
- 10. Build accountability without adding administrative burden.** Consider a family registry, similar to the ACCB, where parents register their child and direct their funding to a specific licensed provider.

At a glance

Measure	Timing	Estimated annual cost
Stabilization	April 1, 2027, until universal funding begins or March 31, 2029	\$0 where enrolment is 75% or more; about \$61M at 70%, \$182M at 60% and \$304M at 50%. About \$103M at the occupancy reported in our survey.
Inclusion	Can begin within the stabilization period; no new legislation needed	Not yet costed. Depends on the number of children assessed and the support rate set.
Universal funding	From April 1, 2028	\$1.24B to \$1.93B, depending on the co-pay and occupancy, drawing on the CCFRI, \$10 a Day and CCOF streams already funded.
Wage certification differential	Phased in over the stabilization period	Not yet costed. Sits outside the benchmark model.

PART 1

Stabilization funding for BC child care, 2027–2029

The spaces were built rapidly. The children are not there to use them.

British Columbia has spent a decade building child care spaces. Licensed capacity has grown 51% since 2018. Over the same period, the number of young children has moved the other way: B.C. has almost 10% fewer children aged three and four than it did in 2018, and that age group shrank in 54 of the province's 59 school districts.

Child population, rather than births, is the measure that matters here, because it captures both the children born in B.C. and the families who arrive with young children. Provincial birth counts have held roughly steady since 2018, but most children now aged three and four were born in 2022 and 2023, when births dipped below 42,000 a year, and slower immigration has added fewer young families to that cohort. B.C.'s total fertility rate fell to 1.00 in 2023, the lowest in Canada.

The decline is not even, and that matters, because child care works at the community level, not the provincial level. Between 2018 and 2026 the number of three- and four-year-olds fell by 25% in Greater Victoria and by 13% in both Richmond and West Vancouver. Each now has more than 150 licensed spaces for every 100 children in this age group.

The trough is temporary. Births rebounded in 2024 and 2025, and those children reach 3-to-Kindergarten rooms between 2027 and 2029, the same period this proposal covers. But immigration is slowing at the same time. The federal 2025–2027 Immigration Levels Plan, announced in October 2024, cut permanent resident targets to 395,000 in 2025, 380,000 in 2026 and 365,000 in 2027, down from 500,000 in the previous plan. That slows the arrival of the young families who have historically filled B.C. programs, and who have disproportionately contributed to birth rates in their communities.

In 2019, the Province funded community child care planning through the Union of BC Municipalities, supporting local governments to inventory existing spaces and project how many would be needed over the coming years. The targets that guided child care expansion were set in good faith, on the population trends of the time. Spaces then grew many times faster than the number of children born or arriving.

More 3-to-5 spaces than children

Between 2018 and 2026, funded spaces for children aged three to school age (group care for 30 months to school age, plus preschool) grew from 51,238 to 77,960, an increase of 52%. Over the same period, the number of children aged three and four in B.C. fell from 94,352 to 85,332, a drop of almost 10%. The three-to-four population fell in 54 of B.C.'s 59 school districts.

B.C. now has **106 child care spaces for every 100 children aged three and four**, up from 65 in 2018. In **32 of 59 school districts**, there are more spaces than children in this age group.

This figure measures the spaces available to this age group in each community. It counts group care for 30 months to school age and preschool, and it also accounts for the share of family child care and multi-age spaces that serve three- and four-year-olds (five spaces per family child care licence and five per multi-age licence). Those mixed-age spaces were routinely unreported, or not factored into the projections in municipal child care needs assessments, so earlier estimates understated the care available. Counting group care alone gives 74 spaces per 100 children; adding preschool gives 91. Family and multi-age allocations fall within the age mix the Child Care Licensing Regulation (Schedule E) permits for each licence type; they are not observed enrolment.

School district	Spaces per 100 children aged 3–4 (2026)	Children aged 3–4, 2018 → 2026
West Vancouver	184	700 → 611
Greater Victoria	154	3,807 → 2,859
Richmond	153	3,749 → 3,243
Okanagan Skaha	144	859 → 712
North Vancouver	129	2,856 → 2,548
Coquitlam	126	4,875 → 4,181
Kamloops–Thompson	119	2,393 → 2,183
Langley	119	3,697 → 3,772
Central Okanagan	116	3,834 → 3,722
Delta	114	2,036 → 1,933

Spaces receiving CCOF/CCFRI funding by school district, 2018 and 2026, with child population estimates by school district, compiled by Child Care Professionals of BC. Family child care and multi-age spaces allocated at five per licence to children aged three and four.

The result is that in much of the province there is now more licensed care for three-to-five-year-olds than there are families to fill it. The oversupply is specific to this age group. Care for children under three remains scarce overall, at 27 spaces per 100 children province-wide, and child care deserts still exist in specific care types and in specific communities, but they are no longer the norm.

Why an oversupply of 3-to-5 care puts infant-toddler care at risk

Infant-toddler rooms are the most expensive care to run. Group size is capped at twelve with a 1:4 ratio, compared with twenty-five children at 1:8 for group care from 30 months to school age.

Operators have kept infant-toddler rooms viable by pairing them with 3-to-5 rooms, whose revenue helps subsidize the infant-toddler program. In our 2026 survey, 57% of operators running infant-toddler care said their 3-to-K or preschool rooms had helped cover its cost.

An oversupply of 3-to-5 care removes that revenue. When the 3-to-5 room is half empty, the infant-toddler room loses the subsidy that kept it open, and the care type B.C. has least of becomes the one most at risk of closing.

Infant-toddler supply has caught up in some communities

Province-wide, infant-toddler coverage more than doubled between 2018 and 2026, from 12 to 27 spaces per 100 children under three. In several school districts it now meets or exceeds international benchmarks: the European Union's original Barcelona target that 33% of children under three participate in formal child care, and in two districts its 2030 target of 45%.

School district	Spaces per 100 children under 3 (2026)	2018
Richmond	46.5	19.0
West Vancouver	46.5	13.0
North Vancouver	41.4	22.5
New Westminster	41.2	19.4
Greater Victoria	39.7	16.0
Langley	37.4	15.6
Maple Ridge–Pitt Meadows	35.4	14.9
Coquitlam	34.7	17.5
Kamloops–Thompson	33.2	14.6

Group infant-toddler spaces plus two spaces per family child care licence and three per multi-age licence, per 100 children aged 0–2. EU benchmarks: 2002 Barcelona target 33%; 2022 Council Recommendation, 45% by 2030.

No coverage target has been set for B.C., either for infants and toddlers or for children under six overall. The Canada–B.C. agreement commits to space counts instead: 30,000 new spaces for children under six by 2025–26 and 40,000 by 2027–28. Eight other jurisdictions agreed to a coverage rate of 5.9 licensed spaces for every 10 non-school-age children, and the CCPA applies that rate to every province as the interim federal benchmark. A space count can be met while coverage falls in a given community, or while spaces land where the children are not. By the measure above, B.C. has about 58 spaces for every 100 children aged zero to four, up from 34 in 2018, and 26 of 58 school districts are at or above 59. Further space creation should be targeted to the communities and care types still below that benchmark.

In the 2026 BC Child Care Operator Survey, **86%** of responding operators reported unfilled spaces in at least one program. **Half (50%)** said that on their current trajectory they can continue operating as they are for two years or less, and **a further 10%** are already planning a closure, a room closure or a significant reduction.

The current funding design turns empty spaces into closures

CCOF base funding, the CCFRI fee reduction and the CCFRI provider payment are all paid per **enrolled child**. A provider's costs are fixed to **licensed capacity**: the lease, licensed staffing ratios, insurance and utilities. A program that loses a quarter of its enrolment loses a quarter of its operating revenue and almost none of its cost. Programs may be able to weather low enrolment for a limited period, but eventually they need to close classrooms or entire facilities.

The only cost a program can cut quickly is staff, and only if enrolment ratios allow. But a program that lays off educators cannot take the next family who comes looking for care. Oversupply then becomes a practical shortage, in a community that did not have one before.

The proposal

Child Care Professionals of BC asks the Province for short-term, temporary stabilization funding, in place until a universal child care model is launched across the province, or until March 31, 2029, whichever comes first.

From April 1, 2027, the Province would pay CCOF base funding, the CCFRI fee reduction and the CCFRI provider payment on **75% of a facility's licensed spaces, or on actual enrolment, whichever is greater**.

Four features keep this a controlled, temporary measure.

A floor, not a ceiling. A program enrolled above 75% is paid on actual enrolment and is better off for every child it adds. The incentive to fill runs in the same direction it does today. The floor only stops a program falling below the point where fixed costs make it unviable. As enrolment improves and stabilization is no longer needed for a particular program, funding reverts to enrolment automatically.

A three-child minimum. Funding applies only where a facility has three or more children enrolled. In B.C., care for two or fewer unrelated children does not require a licence. The threshold confines stabilization to programs that genuinely need a licence to operate and could not lawfully continue as licence-not-required care.

Twelve months' licensed operation. A facility must have held a child care licence at its address for at least twelve months. The measure is meant to stabilize established programs facing an unprecedented enrolment decline, not to carry a new operator through a first year that should have been budgeted for low enrolment in the current market.

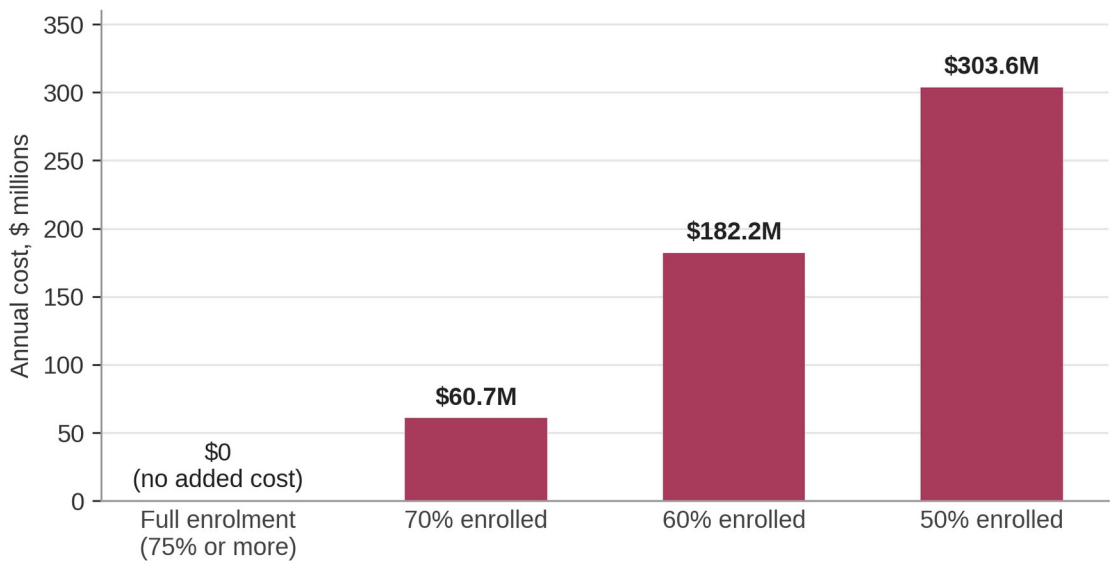
A fixed end date. The measure ends when universal funding is in place, or on March 31, 2029, whichever comes first. It is a bridge across a demographic trough, timed to end as the larger 2024 and 2025 birth cohorts reach preschool age. It is not a permanent change to the funding model, and it commits no government beyond the current fiscal plan period.

The Province already receives everything needed to administer it. Every licensed provider files monthly enrolment reports with the Ministry, and licensed capacity is recorded on each facility’s licence. No new reporting is required. Because the floor applies only below 75%, the funding goes where the enrolment decline is deepest and nothing extra goes to programs that are not severely affected.

What it costs

Modelled across the 126,624 licensed non-school-age spaces in the Ministry’s CCOF dataset:

Annual cost of a 75% stabilization floor, by enrolment level



Enrolment across all facilities	Annual cost	Two-year maximum
Full enrolment (75% or more)	\$0 (no added cost)	\$0
70%	\$60.7M	\$121.5M
60%	\$182.2M	\$364.4M
50%	\$303.6M	\$607.3M

How the scenarios are built: Each assumes every facility is at the stated enrolment. Cost equals full-enrolment operating funding (about \$1.21 billion a year across the 126,624 spaces, implied by current CCOF base and CCFRI rates) multiplied by the gap between enrolment and 75%. This is an approximation; actual cost depends on how enrolment is spread across programs and regions, and on the Ministry’s own enrolment data, which is not published.

Enrolment is not the same everywhere, so the real cost sits between these scenarios. At the occupancy reported in our 2026 survey, which varies by program type, the modelled cost is **\$103.3**

million a year. That is less than half (44%) of the \$233.2 million a year the system is already budgeted to pay at full enrolment and does not currently pay because the seats are empty. The request asks the Province to temporarily direct money toward keeping open the spaces it has already paid to create.

The cost is self-extinguishing. It is the gap between actual enrolment and 75%, so it falls automatically as children return and reaches zero without any further decision by government.

What it protects

The public investment already made. A decade of capital grants, New Spaces funding and federal transfers under the Canada–B.C. agreement built this capacity. A space lost to closure is not recovered cheaply. The lease goes, the licence lapses, the playground is repurposed and the educators are laid off. Rebuilding costs far more than holding. Stabilization protects an asset the Province has already paid for.

Early childhood educators. Staffing is the first cost a struggling program cuts, and an ECE laid off in a contracting sector is often lost to it permanently. B.C. has spent years recruiting and training this workforce through the ECE Wage Enhancement, bursaries and expanded training seats. Stable revenue is what lets a program keep educators employed and retain the staff the sector will need when the larger birth cohorts arrive.

The runway to universal child care, without creating new child care deserts. Much of the \$10 a Day expansion has come from converting existing licensed programs. A program that closes in 2027 cannot be converted to universal child care in 2028. Holding capacity through the trough preserves the inventory any future universal expansion will draw on, and protects the Province's ability to meet its commitments under the Canada-wide agreement.

Infant and toddler care. Infant-toddler rooms are capped at twelve children with a 1:4 ratio, which makes them very costly to build and operate. Providers commonly build them alongside 3-to-5 rooms so the infant-toddler room is financially viable. When the 3-to-5 rooms are under-enrolled, operators may have to close the infant-toddler program simply to stop the losses. If that happens just before rising birth and immigration rates push infant demand back up, infant-toddler child care deserts will reappear across the province.

Care that fits different families. Vacancy is deepest in preschool, group care for 30 months to school age, family child care and multi-age programs. Family and multi-age programs also disproportionately serve part-time families, shift workers, rural communities and children who do not thrive in large group care. When a community loses that variety, it loses the ability to meet its families' child care needs.

Fiscal predictability for government. The measure is capped by its own formula, needs no new administrative apparatus, ends on a fixed date and declines automatically as enrolment recovers. It is an inexpensive, controlled instrument, and far more cost-effective than rebuilding lost capacity.

PART 2

Funding inclusion in BC child care

A child who needs support in licensed child care in British Columbia has few or no options for getting that support. Operators who responded to our 2026 survey are carrying unfilled licensed spaces, and families of children with support needs are still told there is no room. The rooms exist. What is missing is the funding to staff them for the children who need more.

That is a system design problem, and it can be fixed with **three changes that need no new legislation.**

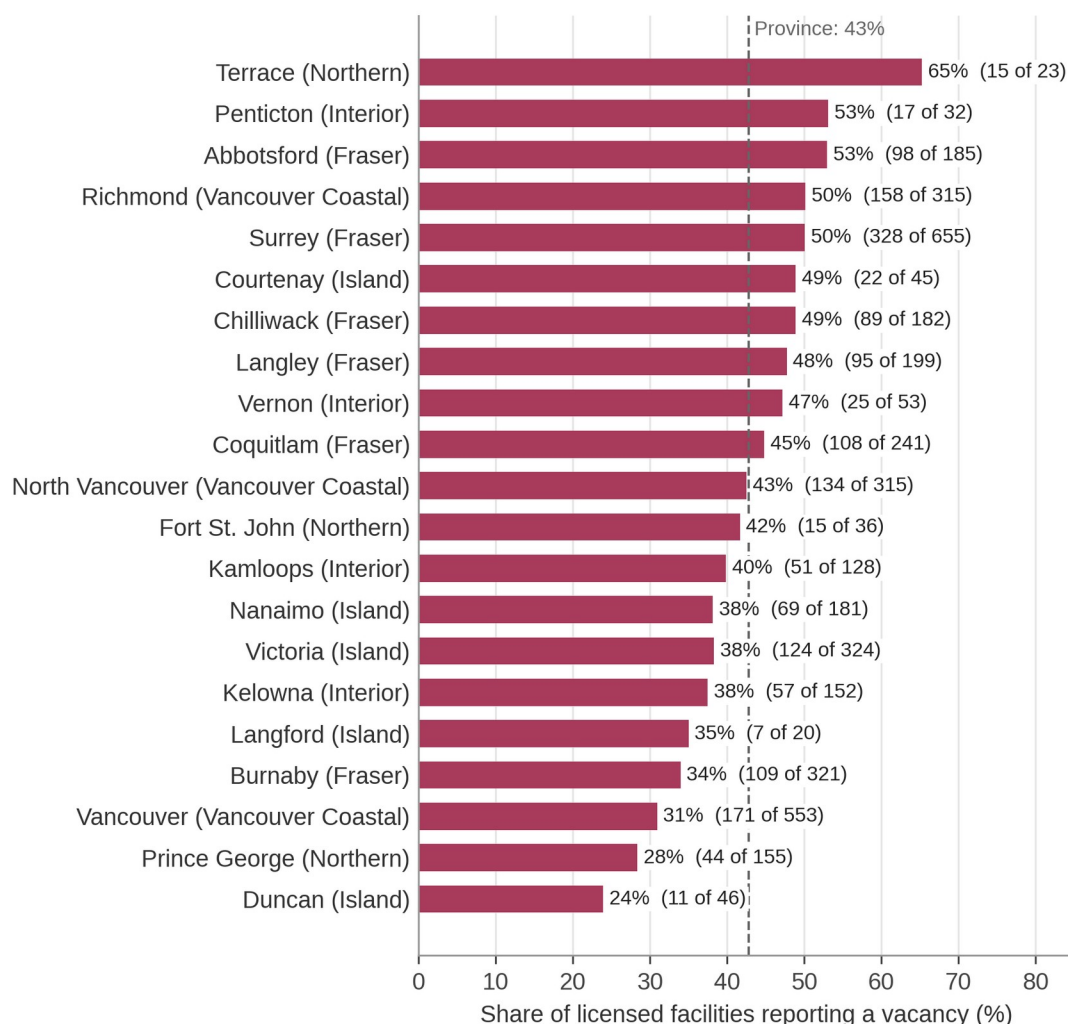
The spaces exist. The support does not.

Operators responding to the 2026 BC Child Care Operator Survey report unfilled licensed spaces across every program type.

Program	Operators reporting unfilled spaces
Preschool	33 (94%)
3-to-Kindergarten	75 (83%)
Multi-age	18 (75%)
Licensed home-based	21 (72%)
Before & after school	27 (55%)
Infant & toddler	29 (41%)

While this survey has limitations, the BC Child Care Map also tracks current vacancies. As of September 23, 2026, **2,567 of the 5,991 licensed facilities on the map (43%)** reported at least one vacancy.

Share of licensed facilities reporting a vacancy on the BC Child Care Map, largest communities in each health authority



Source: BC Child Care Map Data, Ministry of Education and Child Care, retrieved September 23, 2026.

Vacancies on the map are self-reported by operators and are vacancies that are currently available for enrolment; they do not count deliberate vacancies that are not available for enrolment. The map shows whether a facility has a vacancy. Facilities are grouped by mailing locality, because the published data does not identify health authority.

An under-filled room is also where inclusion is cheapest. A 3-to-5 room running at 60% is already staffed for a full room, so the ratios are often favourable. But when a child needs specific 1:1 or 2:1 support, there is no funding to access it. Supported Child Development and Aboriginal Supported Child Development are meant to provide support funding, but both carry long waitlists. Where a placement is offered, it is often for a restricted block of hours and for a prioritized age group, typically four-year-olds. A family paying for a full day of child care receives support for part of the day, and the child can attend only for as long as the support worker is in the room. **This is not inclusion, and it is not universal child care.**

The situation is worse than it first appears.

The new B.C. Children and Youth Disability Benefit is being phased in from April 2026. It provides \$6,500 or \$17,000 a year to families of children with significant disabilities, across all diagnoses. Many children who currently receive Individualized Autism Funding will no longer qualify under the new model, and will lose the individualized funding their families used to pay for individual support workers in their child's program.

The result is a child care system holding staffed rooms that are not funded or supported to provide inclusive care, next to families who are often unable, or, in a \$10 a Day program, not permitted, to fund the support themselves.

What the Province puts behind an included child

The Affordable Child Care Benefit special needs supplement currently pays up to \$150 a month toward the parent's fee.

It is a parent benefit: it offsets the family's fee and never reaches the program as support dollars. It is available only to families already eligible for the Affordable Child Care Benefit, so a family above the income threshold receives nothing, whatever their child needs.

It is also capped. The subsidy and the supplement together cannot exceed the space fee. With fee reductions now bringing parent fees down, and \$10 a Day fees at \$200 a month or less, the supplement is often reduced well below \$150 or disappears entirely.

Once that child reaches kindergarten, the same government funds the same child differently:

Where the child is	Funding per year	Paid to
Licensed child care, special needs supplement	Up to \$1,800, often less (capped at the parent's remaining fee)	The parent, toward their child care fee only; income-tested
Licensed child care, Supported Child Development	Variable hours; waitlisted	The child care provider or SCD agency, for a consultant or limited support staffing
K–12 inclusive education, Level 3	\$12,300	The school district, used as the district determines
K–12 inclusive education, Level 2	\$24,340	The school district, used as the district determines
K–12 inclusive education, Level 1	\$51,300	The school district, used as the district determines

The difference is not only the amount. It is who receives it. School districts are paid directly and spend the money on staffing: education assistants, specialist time, equipment. Child care programs receive nothing they can staff with outside Supported Child Development, and only if the child makes it through the waitlist. A request to enrol a child who needs support is often a request to absorb an

unfunded cost, and potentially to face health and safety incidents that leave the program out of compliance with licensing.

Operators are left choosing between the child, the operating budget and licensing compliance: a choice no child care operator should have to make.

Three changes

1. Issue a Director of Licensing Standard of Practice requiring an inclusion policy, with a provincial obligation attached

The Director of Licensing already uses this instrument, under section 4(1)(e) of the *Community Care and Assisted Living Act*, to require licensees to develop, implement and share written policies. The Active Play and Screen Use policies work exactly this way. An inclusion Standard of Practice would require every licensed program to hold a written policy covering three moments:

- **Enrolment**, where a child arrives with a diagnosed or documented need.
- **Emergence**, where a need becomes apparent after the child has started.
- **Limits**, where the program cannot adequately support the child, and what it must do before withdrawing care.

The policy should not be the operator's obligation alone. At the limits moment, the Standard should trigger a provincial one: a Supported Child Development referral with a published response time, so a family is not left waiting while the program waits with them.

The requirement also matters for a legal reason. Licensed child care is a service customarily available to the public under section 8 of the *Human Rights Code*, so a program owes a duty to accommodate a child with a disability up to the point of undue hardship. Undue hardship is assessed against the resources the provider actually has, and a program with no inclusion funding can reach that point quickly. The duty alone will not produce inclusion; funding is what moves the line.

2. Fund support, pay it to the provider, and remove the income test

Replace the special needs supplement with a support payment made to the operator, set in the range of **\$750 per child per month**, with the goal of eventually matching the levelled funding in place in school districts. Pooled across the three or four children in a program who may be approved for support funding, that makes shared, context-specific support possible, which is exactly how inclusive rooms actually work.

Keep the existing child care special needs assessment as the gate, so no new eligibility machinery is needed. For children in the emergence period, who have no designation yet, SCD consultants can provide short-term, temporary support through consultation or classroom support aide funding.

Remove the income test. A child's support needs do not change with household income, and testing them against it means a family above the threshold has no funded route into care at all.

Recognize the educators who do this work. In 2021, Child Care Professionals of BC proposed a wage enhancement structured by certification, with \$10 an hour for educators holding special needs and infant toddler certification. The certification differential was never built. An educator trained to support a child with complex needs currently receives the same enhancement as one who is not.

The point of the redesign is to change the operator's arithmetic. Enrolling a child with support needs should bring the necessary support, as it is designed to in a school district, rather than bringing a risk that leaves the operator out of compliance with licensing.

3. Create an annual accessibility grant, open to every licensed operator

An annual grant with a clearly published list of eligible items: physical adaptations, sensory and regulation equipment, communication supports, and staff training. The Province's own evaluation of the prototype sites found quality improved where funding was separately targeted to it.

Allocate it by neither auspice nor funding stream. A child's access to an inclusive program should not depend on whether the program is non-profit, private, or enrolled in \$10 a Day.

The Province has said it heard from operators that current funding models limit their flexibility to support high-quality, inclusive care. These three changes are what that flexibility would take.

PART 3

A sustainable funding model for universal child care

There is still no pathway to universal child care in BC

After more than eight years of contract “pilots,” BC still has no stable, inclusive funding model that can deliver universal child care.

The Province funds a universal “prototype,” but \$10 a day reaches only about one in ten children. Proposed by advocates in 2011 and announced as universal in 2018, it has never come close.

Child Care Professionals of BC proposes that the Province stop funding a 2011 price point for a few families and instead fund every licensed centre at a published regional benchmark rate, less a single province-wide parent co-pay. Modelled on funding already committed, this is affordable now. We propose stabilization funding from April 2027 (Part 1) and a universal rollout from April 1, 2028.

Where child care advocates agree

We stand with the Coalition of Child Care Advocates of BC and Early Childhood Educators of BC (ECEBC) in calling for universal child care now. A system available to one in ten children is not universal.

We share their goals: universal access, fair wages for educators, inclusive care, and predictable fees at any program. \$10 a day remains worth reaching, but families cannot wait for it one funding round at a time. A single published co-pay delivers affordable care to everyone now and can be lowered as funding allows.

For parents, this means:

- One price, known before you join a waitlist, at every licensed centre.
- No need to win a spot at a designated site; your child stays with their educators and friends.
- If you move, change jobs or have a second child, the price stays the same.
- Families paying \$28 a day in Kelowna or \$52 in Richmond would pay a co-pay closer to Alberta’s \$15 or Ontario’s \$22.

A 2011 price in a 2026 economy

The \$10 a day figure comes from the *Community Plan for a Public System of Integrated Early Care and Learning*, published by the Coalition and ECEBC in April 2011 after a 2010 draft. It set a vision, not an operating model, and its assumptions have not been updated in 15 years.

Canadian consumer prices rose 38.0% between 2011 and 2026. Adjusted for inflation alone, \$10 a day in 2011 is about \$13.80 a day in 2026 dollars: roughly \$200 a month then, and \$276 a month now, for full-time care. The number was always the slogan; the goal was quality, accessible, inclusive, affordable care, funded sustainably.

The Province has reached the same conclusion

Announcing the Budget 2026 pause, Minister of Education and Child Care Lisa Beare said:

“We are hearing from families that it’s a lottery system. ... We’re hearing from operators that we don’t have the operating model quite right.”

Budget 2026 paused new provider enrolment in \$10 a Day ChildCareBC and the Operating Funding Model for a three-year stabilization period, with \$330 million to hold the system steady. Fees for families already in the program stay at \$10 a day.

What eight years of advocacy did build

The work since 2018 produced something more durable than the slogan: **fee caps and regional affordability benchmarks.**

The Province publishes a benchmark fee by region and care category, updated annually from data covering more than 90% of facilities; it already governs what new providers may charge. That benchmark can carry a sustainable, province-wide funding model using money already spent, just spent differently, rather than waiting for new money in a tight fiscal year.

The funding model

The Province funds child care affordability through four streams: the Child Care Fee Reduction Initiative (CCFRI), \$10 a Day ChildCareBC, the Affordable Child Care Benefit (ACCB), and Child Care Operating Funding (CCOF). No new rate-setting machinery is needed:

Province pays = published regional benchmark – parent co-pay

Modelled against current provincial and federal allocations, excluding the Wage Enhancement, BC could fund every licensed space at its regional benchmark. CCPBC does not speculate on how much of the budget currently reaches spaces directly, but we estimate a parent co-pay of **\$15 to \$23 a day**, comparable to Alberta’s \$15 and Ontario’s \$22.

The ACCB continues underneath, reducing or eliminating the co-pay for the lowest-income families.

The federal government’s July 2026 agreement with BC includes more than \$630 million for affordable child care, recognition that child care underpins the parents who work in health care, mining, retail, the trades and every other sector.

This is no longer a budgeting issue; it is a decision. The Province can keep funding a 2011 price for fewer than one in ten funded spaces, or use the benchmarks and funding it already has to fund every centre at the cost of care.

The proposal: stabilize, then go universal

Phase one — Stabilize, from April 2027. Stabilization funding (Part 1) keeps operators from closing while the Province prepares the transition. It ends when universal funding begins, or on March 31, 2029 at the latest.

Phase two — Universal, from April 1, 2028. Every licensed centre is funded at its regional benchmark less one province-wide co-pay. Government funding varies by care type and region; parents pay the same co-pay everywhere (less any ACCB). Every child, infant to school age, is included: no application rounds, designated-centre lists, lottery or shifting funding models.

Universal child care is not universal unless every child, regardless of ability, can access it. The inclusion changes in Part 2 are part of the universal model, not an add-on to it.

What this gives each party

Families get a predictable price that no longer depends on whether their centre won a funding round.

Providers get a known benchmark rate, regardless of auspice or funding stream, to plan, budget and set wages against.

Educators get stability that improves retention. The ECE Wage Enhancement continues outside the model, and consultation continues, respecting collective bargaining rights, on a publicly available wage grid that sets ECE, ITE and SNE wages by region.

Government gets a model that balances. The benchmark already exists, and the co-pay is a single lever that can be indexed annually, scaled by income, or lowered toward \$10 a day as fiscal room appears, adjusted without disrupting centres and without designing and discarding another funding model.

The system gets real universality, so advocacy and policy work can shift to building quality.

Protecting quality and accountability

Operators are experts at doing more with less, but quality of care must never become the cost of balancing the budget. The Ministry would need to:

- **Secure and grow the Wage Enhancement.** Protect the existing \$6.00 an hour and add \$2.00 an hour each for ITE and SNE certification, building on ECEBC and the Coalition's calls for fair compensation, with a publicly available wage grid set by region. Continue the Professional

Development Bursary. This is the plan's one new cost; it sits outside the benchmark model and can be phased in over the stabilization period.

- **Set operating-hour ranges.** Preschools run 2 to 4 hours; group care for 30 months to school age runs 5 to 9 hours. Benchmarks reflect the 5-hour minimum and rise proportionately per additional hour; hours beyond 9 may carry a fee. Single-educator programs (IHMA, FCC, MA) cap at 8 hours before fees, in line with work-hour and overtime limits. Transparent formulas end centre-by-centre contract negotiation.
- **Embed inclusion.** Children with additional support needs are routinely turned away because operators cannot fund support. Part 2 sets out the urgent changes.
- **Consult on the benchmark formula through 2027–28.** Use the stabilization year to settle a transparent benchmark formula with operators.
- **Build accountability without adding burden.** Operators must be accountable through reporting, but accountability measures should not be excessively onerous or add to the current administrative burden. Consider a family registry, similar to the ACCB, where parents register their child and direct their funding to a specific licensed provider, so the Ministry knows immediately which child each payment supports and the room for fraud is removed. This works under existing systems, does not require parents to disclose their income if they are not requesting ACCB, and removes the need to audit programs on enrolment and CCOF/CCFRI claims.

It's time to act

Operators want to protect the programs families rely on, offer every family affordable care, and keep qualified educators in the field. A funding model that does not reflect the true cost of quality care holds us back, as the Province itself has acknowledged. The stabilization period is the moment to fix it.

BC's operators stand ready to deliver a universal system, but we cannot do it alone. We urge the Government of BC to fund every licensed centre at the true cost of quality care, with one published co-pay every family can plan around.

Every part of this plan comes back to one question: **what is best for the child in the room?** The early years do not wait for the next budget cycle and should not wait for yet-one-more pilot funding model. **We can provide universal care in BC now.**

*Universal. Every child. Every family.
Every educator. Every community.*

Appendix: What universal would cost

Funding every licensed space at its regional benchmark, less a co-pay, would cost the Province roughly **\$1.2 to \$1.9 billion a year**, depending on the co-pay and occupancy.

Parent co-pay (full day)	Every funded space filled	At 75% occupancy
\$15/day	\$1.93B	\$1.45B
\$19/day	\$1.79B	\$1.34B
\$23/day	\$1.65B	\$1.24B

Existing streams available to pool: CCFRI \$754.2M and \$10 a Day \$215.9M (2024–25); CCOF (budgeted figures not available). The ACCB (\$134.4M) stays separate, beneath the co-pay, so families most in need keep access to care. The ECE Wage Enhancement stays outside the model.

Assumptions

- **Spaces:** 181,057 funded licensed spaces (BC government, June 2026).
- **Rates:** 2026–27 affordability benchmarks, weighted by each region's share of spaces.
- **Co-pay:** full-day co-pay × 20 days a month for full-time care.
- **Occupancy:** modelled at 75%, the basis of the stabilization funding formula. Current occupancy ranges from roughly 40% to 80% by program, based on the BC Child Care Map and self-reported vacancies in the 2026 CCPBC operator survey. The Ministry collects actual occupancy through CCOF and CCFRI reporting, but it is not publicly available.

Levers that change the cost

- **Co-pay level:** each \$4 a day moves the cost by about \$105M at 75% occupancy (about \$140M if every space is filled).
- **Payment basis:** paying on enrolment rather than licensed capacity.

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